

Ch 1: Overview & Goals

This 12-week advanced cutting cycle is engineered for experienced athletes (3+ years training, 2+ cycles) seeking maximum fat loss while preserving hard-earned muscle. The protocol integrates multiple compounds at precisely timed dosages, paired with structured cardio and a high-protein caloric deficit.

Advanced cutting cycles differ from beginner approaches by leveraging synergistic compound combinations, ancillary medications, and advanced nutritional periodisation to push past genetic fat-loss limitations.

Parameter	Details	Rationale
	12 weeks (84 days)	Optimal transformation without excessive HPTA suppression
Goal	Fat loss + muscle preservation	Body recomposition during caloric deficit
	Advanced (3+ years, 2+ cycles)	Compound sensitivity established; ancillary knowledge required
Primary Compounds	Test Prop + Anavar + Winstrol	Short ester base + oral hardening agents
	500–700 kcal below TDEE	Aggressive but muscle-sparing with 2.2g/kg protein
Cardio	5× LISS + 2× HIIT per week	Maximum fat oxidation without catabolism
	Nolvadex 40/40/20/20 + Clomid	HPTA restoration within 4–6 weeks post cycle

Ch 2: Week-by-Week Protocol

Testosterone Propionate is the base due to its short half-life, allowing rapid dose adjustment if adverse effects occur. Clenbuterol follows a 2-weeks-on/2-weeks-off protocol to prevent beta-2 receptor desensitisation.

Week	Test Prop mg/EOD	Anavar mg/day	Winstrol mg/day	Clen mcg/day	AI (Anastrozole)	Notes
	100	–	–	20	–	Tolerance assessment
2	100	50	–	40	0.25mg EOD	Introduce Anavar
	100	50	–	60–80	0.25mg EOD	Ramp Clen
5	100	50	–	100	0.25mg E3D	Peak Clen; blood work
	100	50	–	0	0.25mg E3D	Clen off-week
7–8	100	50	50	0–40	0.25mg E3D	Introduce Winstrol
	100	50	50	60–80	0.25mg E3D	Full stack active
11	75	50	50	100	0.25mg E3D	Begin taper
	50	50	50	80	0.25mg E3D	Final week

Ch 3: Nutrition Blueprint

Nutrition must accomplish three simultaneous goals: maintain a caloric deficit for fat loss, provide enough protein to prevent muscle catabolism, and time nutrients to support training performance.

Macro	Amount	Indian Sources	Timing
	2.2–2.5 g/kg BW	Chicken, fish, paneer, egg whites, whey isolate	Every 3–4 hrs; 40g casein pre-sleep
Carbs	1.5–2.0 g/kg BW	Oats, brown rice, sweet potato, banana, roti	Morning + around workouts only
	0.6–0.8 g/kg BW	Eggs, almonds, olive oil, fish oil, ghee	Avoid immediately pre/post-workout
Water	4–5 litres/day	Plain water + electrolytes on heavy cardio days	Sip throughout; 500mL pre-workout

Sample Cutting Day (80 kg, 2000 kcal target)

Meal	Food	kcal	Protein
	4 egg whites + 1 whole egg + 1 roti	320	30g
9:30 AM	30g whey isolate + 100g curd	220	38g
	200g chicken + $\frac{3}{4}$ cup brown rice + sabzi	480	48g
3:30 PM (pre-workout)	Banana + 5g creatine + black coffee	120	2g
	30g whey isolate + $\frac{1}{2}$ cup oats	320	32g
8 PM (dinner)	150g fish + large salad + 1 roti	380	36g
	30g casein / 150g cottage cheese	180	28g
TOTAL		2,020 kcal	214g

Ch 4: Cardio & Training

Cardio during a cutting cycle is a precision tool, not a punishment. Maintain training intensity (load) while reducing volume 20–30% — this signals the body to preserve muscle during the deficit.

Cardio Type	Frequency	Duration	Intensity	Timing
	4×/week	45–55 min	60–65% MHR	Pre-breakfast AM
LISS moderate cardio	1×/week	45 min	65–70% MHR	Any time
	2×/week	20–25 min	85–95% MHR	Post-weights; not consecutive days

Day	Session	Key Lifts	Cardio
	Push	Bench 4×6, Incline DB 3×8, OHP 3×8, Laterals 3×12	HIIT 20 min post
Tuesday	Pull	Deadlift 4×5, Weighted pull-up 4×6, Cable row 3×10	LISS 45 min AM fasted
	Legs	Squat 4×6, Leg press 3×10, RDL 3×10, Leg curl 3×12	LISS 45 min AM fasted
Thursday	Rest	Mobility, foam rolling, stretching	Optional 30 min walk
	Push B	DB shoulder press 4×8, Cable fly 3×12, Dips 3×10	HIIT 22 min post
Saturday	Pull B + Legs B	Barbell row 4×6, Lat pulldown 3×10, Split squat 3×10	LISS 50 min AM
	Rest	—	LISS 45 min walk (optional)

Ch 5: Side Effects & PCT

Proactive side-effect management is the difference between a successful cycle and a health-compromising experience. Implement all countermeasures from day one, not after problems appear.

Side Effect	Compound	Prevention	Treatment
	Test Prop, Anavar	Benzoyl peroxide wash daily; zinc 30mg	Lower dose; oral antibiotics if severe
High E2 / Gyno risk	Test Prop	Anastrozole 0.25mg EOD; monitor blood work	Nolvadex 40mg/day; add Letrozole if severe
	Anavar, Winstrol	TUDCA 500mg + NAC 600mg daily	Discontinue oral; run liver panel
Joint dryness	Winstrol	Fish oil 4g/day; glucosamine 1.5g/day; BPC-157	Reduce dose; add collagen peptides
	Clenbuterol	Taurine 5g/day; slow ramp; BP check weekly	Reduce or stop Clen; consult doctor if > 150/95
Insomnia	Clenbuterol	AM dosing only; last dose before noon	Melatonin 3mg; magnesium glycinate 400mg
	Winstrol, Anavar	Omega-3 4g/day; regular cardio	Blood test; consider discontinuing if HDL < 30

Post-Cycle Therapy (PCT) — Begins 3 Days After Last Test Prop Injection

PCT Week	Nolvadex	Clomid	Support Stack
	40 mg/day	100 mg/day	Ashwagandha 600mg + Zinc 30mg + D3 5000IU
Week 2	40 mg/day	100 mg/day	Continue full stack + Tongkat Ali 400mg
	20 mg/day	50 mg/day	Continue all; add Boron 10mg
Week 4	20 mg/day	50 mg/day	Continue through recovery

Ch 6: Blood Work & Health Monitoring

Blood work is the most important safety practice in any anabolic cycle. Without objective data, you are flying blind. Run a full panel before, during, and after every cycle.

Test	Pre-Cycle	Week 6	Post-PCT Week 4	Red Flag Value
	Baseline	Check suppression	> 400 ng/dL required	< 300 ng/dL post-PCT
Estradiol (E2)	Baseline	15–35 pg/mL target	Should normalise	> 50 or < 10 pg/mL
	Should be normal	Will be near zero (expected)	Must be rising	Flat at PCT week 4 = extend PCT
ALT / AST (liver)	Baseline	< 2× upper limit	Near baseline	> 3× ULN: stop orals immediately
	< 48%	< 52%	Returning to baseline	> 54%: donate blood; risk of clot
Total Cholesterol / HDL	Baseline	HDL > 40 mg/dL	Recovering	HDL < 25 mg/dL: evaluate cardiovascular risk

Key Takeaways — Advanced Cutting Cycle

- Test Prop base allows fast adjustments — this is a key advantage for an advanced cut
- Never start a cycle without having Nolvadex AND an AI on hand before day 1
- Clen is a tool, not a magic fat-burner — nutrition is still 80% of fat loss results
- Winstrol + joint dryness is predictable — BPC-157 and fish oil are mandatory from day 7
- Post-PCT blood work at week 4 is the only way to confirm HPTA has recovered
- Minimum 16 weeks off cycle before starting the next one (minimum: time on = time off)